

ENGLISH FOR WORK / SEPTEMBER 2026

Healthcare Administration English



Teacher's guide

Teach practical workplace communication with specific cases, clear language targets, and a repeatable feedback process.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For hospital administrators, clinic managers, practice administrators, care coordinators, patient-experience leaders, revenue-cycle staff, operations managers, and healthcare-adjacent professionals.

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Patient Access, Scheduling, and Referrals

Clarify missing information before responding.

02 Revenue Cycle, Coding, and Denials

Explain a useful distinction in plain English.

03 Patient Flow, Capacity, and Staffing

Distinguish completed work from pending work and explain its impact.

04 Quality, Safety, and Accreditation

Separate observations, assumptions, and conclusions.

05 HIPAA, Privacy, and Information Governance

Make a request with a clear action, purpose, and realistic timing.

06 Patient Experience and Service Recovery

Acknowledge a communication problem and give a corrected message.

07 Population Health and Care Coordination

Give a handoff that the receiving person can confirm.

08 Executive Dashboards and Board Updates

Explain a useful distinction in plain English.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop to choose a vocabulary, grammar, role-play, or new-dialogue activity.

A 60-minute lesson that works

Prepare the case and the learner pages. Keep the model response hidden until learners have attempted their own. Choose two field terms that are important to understanding the case.

0-5 minutes | Activate experience

Ask learners to name a comparable situation without sharing confidential details. Introduce the communication goal.

5-12 minutes | Read and sort facts

Read the case. Learners identify confirmed facts, unknowns, the audience, and the immediate communication need.

12-22 minutes | Notice and practice language

Explain the workshop pattern. Work through the vocabulary and editing example, then discuss both language checks and the reasons behind the choices.

22-35 minutes | 05 • Conversations

Read one ten-turn conversation aloud, notice a useful expression, then compare the other two. Identify each speaker's purpose and how the exchange ends.

35-47 minutes | 06 • Say it

Rehearse both speaking scenarios from the workbook. Give two minutes of preparation, switch roles, and use the success checks. Add the complication only after a successful first round.

47-55 minutes | Compare and revise

Reveal the model. Discuss alternative acceptable wording. Give one meaning correction and one language correction, then repeat.

55-60 minutes | Retrieve and transfer

Close the materials. Learners recall two expressions and identify one communication habit to use in another situation.

Adapt the session

For 45 minutes, study one complete conversation and rehearse one scenario; assign the others for oral follow-up. For 90 minutes, rehearse all three conversations and give both scenarios a second round. More time should produce more meaningful practice, not longer explanations.

LESSON 01 / FACILITATOR NOTES

Patient Access, Scheduling, and Referrals

Clarify missing information before responding.

CASE

A referral is received without the requested specialty or reason for the appointment. The patient is expecting a booking confirmation.

Teach this move

Name the exact word, fact, or requirement you need clarified. Ask one focused question at a time. Repeat your understanding and give the other person a chance to correct it.

Vocabulary to check: referral, eligibility, authorization, no-show.

Model response

Could you confirm the specialty and referral details through the approved channel? I can explain the scheduling process to the patient, but I cannot confirm an appointment until the required information is available.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. After an introductory phrase such as 'Could you explain', the embedded question uses statement word order: "what this means".

Second-round challenge

The other person uses an ambiguous word such as 'ready', 'approved', or 'urgent'. Clarify it before continuing.

Language-check answers

1. C - Could you confirm when the review starts? The embedded question uses subject then verb: "the review starts".
2. B - Has the report been approved, or is it ready for review? This question distinguishes two meanings of 'ready' that affect the next action.

05 • Conversations

1. A referral without a specialty - Fictional scheduling call: a referral lacks the requested specialty and appointment reason, while the patient expects confirmation.
2. Eligibility checked, authorization still unknown - Separate fictional access case: an eligibility check is complete for a scheduled service, but authorization status is blank.

3. A cancelled visit counted as a no-show - Separate fictional appointment review: a cancellation message arrived before the visit, but the appointment is marked no-show; receipt time is recorded.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | Two referral entries, one uncertain request

A fictional scheduling queue contains two referrals with the same patient reference and date but different originating departments. Neither entry says whether it replaces the other. Discuss what must be clarified before booking.

Role A: Referral coordinator: ask whether these are duplicate or separate requests.

Role B: Referring-office administrator: verify the intended requests with the clinical team without choosing a specialty yourself.

Possible opening: These entries may be duplicates, but the different departments mean I need confirmation before booking.

Success checks

- Do not infer duplication from matching dates alone.
- Ask which request remains active and what service is intended.
- Agree who returns the verified scheduling instruction.

Add a complication: A colleague proposes deleting the later entry to shorten the queue.

LESSON 02 / FACILITATOR NOTES

Revenue Cycle, Coding, and Denials

Explain a useful distinction in plain English.

CASE

A dashboard groups rejected submissions and denied claims together. A manager wants to understand where follow-up work is needed.

Teach this move

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Vocabulary to check: claim, coding, denial, clean claim.

Model response

These categories describe different points in the process. Let's separate submissions that did not enter processing from claims that received a denial, then identify the follow-up required for each group.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised sentence names the action in familiar words. Specialist vocabulary is useful when it adds precision, but repeating abstract nouns can hide the meaning.

Second-round challenge

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Language-check answers

1. B - The backlog is the work still waiting to be completed. This gives the meaning in familiar words.
2. A - How would you describe the next step in your own words? An open request lets the listener show their understanding.

05 • Conversations

1. Rejected submissions and denied claims need different queues - Fictional revenue-cycle review: the dashboard combines submissions rejected before processing and claims denied after review.
2. A clean claim is not a payment guarantee - Separate fictional billing meeting: twenty submissions pass the team's completeness checks, but payer outcomes are not available.

3. Denial count and denial value tell different stories - Separate fictional dashboard exercise: ten denied claims total 1,000 units of currency; two other denied claims total 8,000, with no recovery estimates supplied.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A returned claim and a suspected coding problem

A fictional submission was returned for a missing identifier. A manager calls it a coding denial, but no adjudication response or coding review exists. Explain the known processing issue and route the next action.

Role A: Billing analyst: distinguish the returned submission from an established coding denial.

Role B: Department administrator: ask what needs correcting and what remains unassessed.

Possible opening: The available response identifies a missing identifier; it does not establish a coding denial.

Success checks

- State the observed response without adding a coding conclusion.
- Separate correction of submission data from review of clinical coding.
- Agree the appropriate queue and a verified status label.

Add a complication: The manager wants the item included in a coding-error presentation that afternoon.

LESSON 03 / FACILITATOR NOTES

Patient Flow, Capacity, and Staffing

Distinguish completed work from pending work and explain its impact.

CASE

A unit reports 24 occupied beds and two planned discharges. The discharge timing is unconfirmed, but another team assumes two beds are available now.

Teach this move

Lead with the current state, then the important change or open item. Use completed-action language only for completed work. Add a next update point when it is known; do not invent a date to make the message sound complete.

Vocabulary to check: census, bed management, throughput, staffing ratio.

Model response

There are 24 occupied beds. Two discharges are planned, but their timing is not confirmed. I'll distinguish current availability from expected availability in the capacity update.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision gives observable progress. A positive label alone does not tell the reader what is complete or still needed.

Second-round challenge

The listener asks you to reduce the update to two sentences. Preserve the most important completed and pending items.

Language-check answers

1. A - The team has completed the review. 'Has completed' states that the action is finished.
2. C - Delivery is planned for Friday; the carrier has not confirmed it. This explicitly labels the date as planned and identifies the missing confirmation.

05 • Conversations

1. Two planned discharges are not two available beds - Fictional capacity call: the unit has 24 occupied beds and two planned discharges whose timing is unconfirmed.
2. Physical space without confirmed staffing - Separate fictional capacity exercise: a room is prepared, but staffing coverage for an additional admission has not been confirmed.

3. A throughput update with an unresolved transport step - Separate fictional flow exercise: discharge paperwork is complete for one planned departure, but transport arrival has not been confirmed.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A census snapshot taken at a different time

In a fictional morning briefing, the overnight report shows eighteen occupied beds at 06:00 and the live board shows twenty at 08:00. Movement records have not been reconciled. Discuss the update without treating either count as an error.

Role A: Capacity analyst: explain the snapshot times and request reconciliation.

Role B: Unit coordinator: identify the need to check movements before updating the executive report.

Possible opening: These counts were taken two hours apart, so the difference does not by itself establish an error.

Success checks

- State both counts with their times.
- Separate a possible movement difference from a confirmed data error.
- Agree who verifies the current census before it is circulated.

Add a complication: The chair asks you to use the lower number because it looks better.

LESSON 04 / FACILITATOR NOTES

Quality, Safety, and Accreditation

Separate observations, assumptions, and conclusions.

CASE

A near-miss report identifies a scheduling error caught before the appointment. The review has not established how the error occurred.

Teach this move

Say what the evidence shows, identify what is still unknown, and limit the conclusion to that evidence. Words such as 'may', 'suggests', and 'in this sample' are useful when the uncertainty is real. Do not soften an urgent, verified safety concern.

Vocabulary to check: near miss, accreditation, audit trail, corrective action.

Model response

The error was identified before the appointment. The cause is still under review. I'll record the sequence of events and the checks that caught it without assigning blame or an unconfirmed cause.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision states the finding and its scope. It avoids turning limited evidence into a universal claim.

Second-round challenge

Someone asks for a yes-or-no answer when the evidence supports only a qualified answer. Be concise while preserving the uncertainty.

Language-check answers

1. C - The change may have contributed to the delay. 'May have contributed' presents a possibility without claiming the cause is established.

2. B - In this pilot, the observed result was ... This locates the finding in the setting actually studied.

05 • Conversations

1. An intercepted scheduling error with no established cause - Fictional safety review: a scheduling error was caught before the appointment, and the cause remains unknown.

2. Missing evidence is not proof the check never happened - Separate fictional accreditation preparation: three sampled files lack a documented administrative check; whether the checks occurred is unknown.

3. Fewer reports do not automatically mean fewer errors - Separate fictional quality meeting: near-miss reports fell from twelve to five after the reporting portal changed; reporting behavior has not been assessed.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A repeated error with different booking routes

Two fictional scheduling errors occurred this week, one through an online form and one by telephone. The final wrong department was the same. Discuss what can be concluded before a common root cause has been assessed.

Role A: Quality reviewer: state the shared outcome and different routes.

Role B: Scheduling manager: propose evidence gathering without declaring a single cause.

Possible opening: The errors reached the same department, but they entered through different routes.

Success checks

- Separate a common outcome from a demonstrated common cause.
- Request the relevant audit trails and source instructions.
- Keep corrective-action effectiveness unconfirmed until reviewed.

Add a complication: A colleague wants to close both cases under a single untested corrective action.

LESSON 05 / FACILITATOR NOTES

HIPAA, Privacy, and Information Governance

Make a request with a clear action, purpose, and realistic timing.

CASE

A colleague requests a full patient list for a limited administrative task. The appropriate scope and access have not been confirmed.

Teach this move

State the action and the information needed. Explain why it matters and specify a deadline only when one is given or agreed. 'Could you' is polite; repeated apologies can obscure a legitimate request. Urgent situations may require a direct request.

Vocabulary to check: HIPAA, protected health information, minimum necessary, business associate.

Model response

Could you specify the purpose and fields required? Let's confirm the permitted access and necessary information through our privacy process before sharing the list.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision names the document and requested response. It avoids expressions that may be unfamiliar or ambiguous for an international audience.

Second-round challenge

The other person cannot meet the requested timing. Clarify an alternative without inventing authority to change a formal deadline.

Language-check answers

1. B - Send it no later than Thursday. 'By' sets the latest point, while 'after' refers to a later time.
2. A - Could you confirm the document version needed for the review? The requested action and information are explicit.

05 • Conversations

1. Request the task scope before the full patient list - Fictional administrative request: a colleague asks for a full patient list for a limited task, and access scope is unconfirmed.
2. A vendor requests a sample file - Separate fictional privacy exercise: an unverified external vendor requests a patient-data sample for a demonstration; no sharing approval exists.

3. Remove identifiers without claiming anonymity - Separate fictional reporting exercise: a draft administrative extract omits names but retains patient numbers and detailed appointment histories.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | An old folder permission follows a new role

A fictional administrator has moved to another department but can still open a patient-report folder used in their previous role. No new access need has been established. Hold a call requesting an access review without opening additional records.

Role A: Department administrator: report the remaining access and request confirmation of the appropriate permission.

Role B: Access-control specialist: ask for the folder reference and new role through the approved channel.

Possible opening: My previous patient-report access still works, but I need you to review whether it fits my new role.

Success checks

- State the access issue without browsing unnecessary patient records.
- Request a role-specific permission review.
- Agree a responsible reviewer and avoid claiming a legal conclusion.

Add a complication: A coworker suggests retaining access in case it becomes useful later.

LESSON 06 / FACILITATOR NOTES

Patient Experience and Service Recovery

Acknowledge a communication problem and give a corrected message.

CASE

A patient has called twice about a delayed response. The administrator can investigate the communication delay but cannot make a clinical decision.

Teach this move

Name what was unclear or wrong, acknowledge its effect, and correct it. A brief apology can help, followed by a practical next step. Avoid explaining away the other person's reaction or promising an outcome you cannot control.

Vocabulary to check: grievance, service recovery, HCAHPS, patient experience.

Model response

I'm sorry you have had to call again. I can check the status of your request and confirm the next contact. For the clinical question, I'll connect you with the appropriate care team.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision takes responsibility for the message without blaming the listener. It identifies what needs to be corrected.

Second-round challenge

The listener remains disappointed after the correction. Acknowledge the impact and restate the available next step calmly.

Language-check answers

1. A - I'm sorry my earlier wording was unclear. This acknowledges the speaker's contribution to the problem.
2. C - The corrected date and the current confirmation status. A repair needs usable corrected information, including any uncertainty.

05 • Conversations

1. Repairing a promised callback message - Fictional service-recovery review: a patient has called twice about a delayed response, and the administrator cannot make clinical decisions.
2. Correcting a guaranteed appointment message - Separate fictional patient-experience case: an administrator said an appointment was guaranteed, but the record shows only a requested slot.

3. A survey score is not this person's resolved concern - Separate fictional service review: a department cites an improved HCAHPS score while one patient's reported communication concern remains unresolved.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | Directions to the wrong entrance

A fictional appointment reminder directed visitors to Entrance A, which is closed for this training case; the confirmed open entrance is B. A patient missed check-in while looking for A. Discuss a corrective phone response without promising clinical rescheduling.

Role A: Patient access supervisor: acknowledge the incorrect directions and provide the verified entrance information.

Role B: Patient liaison: ask how the appointment question will reach the appropriate scheduling team.

Possible opening: Our reminder gave the wrong entrance; Entrance B is the confirmed route, and I'm sorry for the inconvenience.

Success checks

- Accept the communication error without blaming the patient.
- Give the verified correction and distinguish it from rescheduling authority.
- Agree who reviews the appointment status and contacts the patient.

Add a complication: A colleague wants to describe the patient as a no-show without reviewing the circumstances.

LESSON 07 / FACILITATOR NOTES

Population Health and Care Coordination

Give a handoff that the receiving person can confirm.

CASE

A discharge coordination note lists transport as pending. The next coordinator needs to know who is following up and what has been confirmed.

Teach this move

State the current situation, relevant background, open task, and receiving role. Ask for acknowledgment or repeat-back of the critical item. Sending a message and transferring responsibility are not always the same thing.

Vocabulary to check: care gap, readmission, discharge planning, social determinants.

Model response

Transport remains unconfirmed. The request is recorded, and the assigned coordinator is following up. Please check the latest status with that person and record any change before telling the patient a departure time.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised sentence asks for an identifiable receiving team instead of leaving responsibility with an unspecified 'someone'.

Second-round challenge

The intended recipient cannot take ownership. Keep the status clear and identify the need for an authorized reassignment.

Language-check answers

1. C - Please confirm who will complete the outstanding check. It asks for an identifiable person or team responsible for the task.
2. B - So the result is pending and your team will follow up; is that correct? This repeats the status and responsibility and invites correction.

05 • Conversations

1. Transport pending at the coordination handoff - Fictional discharge-coordination handoff: the note lists transport as pending, with no confirmed collection time.

2. A care-gap list is not a confirmed missed service - Separate fictional population-health handoff: a dashboard flags six possible care gaps, but outside-provider records have not been reconciled.

3. Handing over an unresolved access barrier - Separate fictional coordination exercise: a person reports difficulty reaching follow-up appointments; the specific barrier and available assistance are unconfirmed.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | A follow-up referral sent but not received

In a fictional discharge-coordination case, an outgoing referral was sent yesterday. The receiving service has not acknowledged it, and no appointment is confirmed. Practice transferring follow-up responsibility at shift change.

Role A: Outgoing coordinator: distinguish transmission from acceptance and hand over the reference.

Role B: Incoming coordinator: confirm ownership of checking receipt and appointment status.

Possible opening: The referral was sent yesterday, but receipt and appointment arrangements are still unconfirmed.

Success checks

- Separate sent, received, and scheduled statuses.
- Identify the receiving owner and unresolved questions.
- Avoid promising an appointment while agreeing a clear follow-up action.

Add a complication: The family asks you to say that follow-up is fully arranged.

LESSON 08 / FACILITATOR NOTES

Executive Dashboards and Board Updates

Explain a useful distinction in plain English.

CASE

A board chart shows waiting time falling from 40 to 30 minutes, but the reporting period changed from monthly to weekly.

Teach this move

Start with the reader's question. Explain one unfamiliar term with familiar words, then give a relevant example or contrast. Check understanding without asking only 'Do you understand?'.

Vocabulary to check: dashboard, KPI, variance, owner.

Model response

The reported waiting time is lower, but the periods differ. I'll label the time windows and prepare a comparable view before we describe the change as a sustained improvement.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised sentence names the action in familiar words. Specialist vocabulary is useful when it adds precision, but repeating abstract nouns can hide the meaning.

Second-round challenge

Your listener is new to this field. Replace two specialist expressions with clear explanations without changing the meaning.

Language-check answers

1. B - The backlog is the work still waiting to be completed. This gives the meaning in familiar words.
2. A - How would you describe the next step in your own words? An open request lets the listener show their understanding.

05 · Conversations

1. A monthly wait compared with a weekly wait - Fictional board preparation: a waiting-time chart falls from 40 to 30 minutes, but the reporting interval changed from monthly to weekly.
2. Target variance versus change from last month - Separate fictional dashboard: response time is eight days, the target is six, and last month's result was ten.

3. A department average hides different queues - Separate fictional executive exercise: one queue averages ten minutes and another fifty; the combined figure lacks queue volumes.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | Completed cases versus all cases

A fictional board dashboard reports response time only for completed requests. Twenty requests remain open and are excluded; their waiting times are not supplied. Explain why the completed-case figure does not describe the whole queue.

Role A: Performance analyst: explain the denominator and the missing open-case view.

Role B: Executive sponsor: ask what additional measure is needed before drawing a conclusion.

Possible opening: This KPI covers completed requests only; the twenty open requests are outside its calculation.

Success checks

- State the population included in the KPI.
- Avoid inventing waiting times for the open cases.
- Agree who provides a separate open-queue measure.

Add a complication: A board member asks you to count the open cases as having zero waiting time.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and writes a 70-110 word message. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional language practice for professional communication. Clinical, safety, regulatory, and patient-care decisions follow qualified supervision and current local procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

HHS: HIPAA business associates

<https://www.hhs.gov/hipaa/for-professionals/privacy/guidance/business-associates/index.html>

AHRQ: SBAR communication tool

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/sbar.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.